AUTHORITY BLUEPRINT™

Hedges Dental

Dr. Jeffrey Hedges, DMD

Louisville, Kentucky

Prepared exclusively for Dr. Jeffrey Hedges

March 2026 | Confidential

Under the direction of Roger, Director — Authority Systems Group™

Authority Systems Group™

authoritysystemsgroup.com

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Roger, Director — Authority Systems Group™

Jeff — you have the clinical skills, the location, the community reputation, and the second domain that most dentists attempting the implant transition would kill for. What you don't have yet is a system that takes all of that and makes it visible to the specific patients who are six months into researching who they're going to trust with a $30,000 decision. That's what this Blueprint builds.

The immediate opportunity is your dormant treatment queue. Your practice management software contains patients who were diagnosed with an implant need, said "let me think about it," and then got busy and disappeared. They didn't decline — they deferred. Conservatively, 20–50 patients matching this profile are sitting in your records right now. A three-touch reactivation sequence targeting them specifically is the single fastest path to a booked implant case — and it can launch this week, before any website is rebuilt or any ad is placed.

The second opportunity is the ClearChoice gap. ClearChoice Dental Implant Center sits in your ZIP code on Viking Center Drive. They spend more on advertising in a month than most private practices spend in a year. They dominate awareness for full-arch implants in Louisville. But they charge $52,000 on average for cases that comparable private practices do for $30,000–$45,000. They don't accept insurance. They average 3.2 stars on consumer review platforms, with the most common complaint being financial transparency. Every one of those weaknesses is a positioning opportunity that you are currently not exploiting. The 30–40% of full-arch patients who've already been to a ClearChoice consultation and walked out sticker-shocked are actively searching for an alternative right now. You are that alternative. They just don't know it yet.

The third opportunity is toplouisvilledentist.com. You already own this domain. There's already an All-on-4 page on it. The strategic instinct is correct — and it is ahead of 95% of general dentists attempting this transition. The problem is that the site is not developed to the point where a patient three months into researching their options would feel confident enough to book. We rebuild it as a dedicated implant authority conversion site in 30–45 days, and it becomes your primary implant lead pipeline for the next decade.

90-Day Targets:

- Dormant patient reactivation deployed: all identified implant-diagnosed, non-converted patients contacted
- New implant consultations from reactivation: 3–8
- toplouisvilledentist.com rebuilt and live: yes
- Google Business Profile fully optimized for "dental implants Louisville KY": yes
- Google reviews: baseline → 25+ (implant-specific reviews prioritized)
- Nelson & Lowy referral partnership conversation initiated: yes
- CareCredit financing callout live on all implant pages: yes

- Fast Cash Sprint revenue: $18,000–$75,000 (see Section 6 for methodology)

12-Month Vision: Dr. Jeffrey Hedges is Louisville's recognized private-practice implant dentist — the name that surfaces when anyone searches "dental implants Louisville," whose patient transformation stories circulate through 10 Nextdoor neighborhoods, and whose referral pipeline from Nelson & Lowy and area physicians produces consistent full-arch consults every week. He is doing 8–12 implant cases per month. He has dropped two insurance contracts he no longer needs. His treatment coordinator closes at 65%+. His schedule looks different than it did 12 months ago.

Start here, in this order:

14. Section 6 — Fast Cash Sprint™. These are the moves you execute this week and next. They generate early proof and early cases.
15. Section 9 — toplouisvilledentist.com rebuild brief. Scope this with Christy this week so the build can start immediately.
16. Section 10 — Email sequences. The reactivation sequence (Sequence 1) is fully written and ready to deploy.

— Roger, Director | Authority Systems Group™

Roger, Director — Authority Systems Group™

Jeff — here's how I see your situation, and why we're sequencing it the way we are.

You're a credentialed implant dentist with an established Louisville practice, a UofL faculty appointment, a Nextdoor Neighborhood Favorite designation across ten neighborhoods, and a legitimate community trust signal that most dentists in this market haven't earned after 20 years of trying. The problem isn't your work. The problem isn't even your marketing budget. The problem is that every digital asset you control is telling the market you're a friendly neighborhood family dentist — which is exactly the wrong message to send to a patient who has been researching a $35,000 full-arch case for four months and is trying to decide who is serious enough to trust.

The marketing audit we ran on hedgesdental.com and toplouisvilledentist.com put you at a 22/100 — a critical score. That is not a judgment on the quality of your clinical work. It is a measure of how much of your actual capability is invisible to the people who are looking for exactly what you do. The

gap between what you can do and what the market thinks you do is the entire opportunity this Blueprint is designed to close.

Here's what the data tells us that matters most right now:

The ClearChoice problem is actually a ClearChoice gift. They're in your ZIP code. They're spending heavily. They're charging $52k average for full-arch cases and getting 3.2 stars on consumer review platforms. The patients who've been through their consultation and left with sticker shock are currently back on Google looking for a private practice alternative. You are positioned to be that answer — if your digital presence reflects what you can actually do. Right now, it doesn't.

The toplouisvilledentist.com domain is your most valuable underutilized asset. You had the instinct to acquire it and build an All-on-4 page. That instinct is correct. Most general dentists attempting the implant transition don't have that domain or that page. The problem is the execution — one-paragraph service descriptions do not convert a patient who has been researching for three months. We rebuild it into a full conversion system: educational content, patient stories, financing visibility, direct ClearChoice counter-positioning, and a free consultation offer. That site, properly built, becomes your primary implant lead engine.

Your Nextdoor reputation is a dormant asset. Neighborhood Favorite in 10 Louisville neighborhoods — Bancroft, Graymoor-Devondale, Brownsboro Farm, Briarwood, and others. That kind of community trust takes years to earn and cannot be manufactured. It is currently generating zero implant cases because there's no strategy behind it. We change that this month.

The CareCredit situation needs to be fixed this week. You're a registered CareCredit provider. It appears on their doctor locator. It appears nowhere on your websites. The number one reason implant patients don't book is cost fear — not actual cost. When a patient can see "$45,000 full-arch case = $850/month," the entire conversation changes. That callout needs to be on every implant page before anything else.

The sequencing of this Blueprint is deliberate:

17. **Fast Cash first** — because your dormant patients are the fastest path to cases and we want you seeing results within 30 days. A single full-arch case this month pays for everything.
18. **Website and positioning second** — because ads and SEO poured into a site that positions you as a family dentist will produce family dentist patients. Get the foundation right before you amplify.
19. **Content engine and referral system third** — because these are the compounding assets that run on their own by month six.

For this to work, I need three things from you:

20. **Your patient list pulled by open treatment plans** — specifically any patient who was diagnosed with implant need (single or arch) and never converted. Christy should be able to pull this from your practice management software in under an hour.
21. **One hour with me or the team each month** — for a strategy call where we review what's working and adjust.
22. **Content approval turnaround of 48 hours** — for any copy we send you for review. The bottleneck in these engagements is almost always client approval speed. Forty-eight hours keeps everything moving.

Let's get to work.

— Roger, Director | Authority Systems Group™

Daniel Frost, Chief Strategy Officer — Authority Systems Group™

3A. Market Context — Louisville Implant Dentistry

Louisville, Kentucky is a metro of approximately 800,000 residents (Jefferson County and surrounding suburban counties). Jefferson County's dental market is mature, competitive, and in a state of rapid consolidation — corporate dental groups have opened multiple locations in the Louisville corridor in the past 36 months, and specialist referral networks have traditionally controlled the premium implant case flow.

The Louisville implant dental market has the following structural characteristics relevant to Dr. Hedges' positioning:

High corporate market penetration. ClearChoice Dental Implant Center operates at 9351 Viking Center Drive — in the same 40222 ZIP code as Hedges Dental. Corporate dental groups including Heartland Dental, Aspen Dental, and regional DSO operators have established multiple locations

throughout Jefferson County and the eastern Louisville corridor. These operations market on convenience and brand recognition, not on the quality of the individual doctor-patient relationship.

Specialist default routing. Louisville's general dentistry ecosystem routes implant cases to oral surgeons and periodontists by default. Oral Surgery Consultants of Kentucky, Innovative Oral Surgery (4.9–5.0 star rating across locations), and similar groups have established strong referral pipelines that intercept patients before they ever reach a private-practice general dentist with implant capability. This default routing is not based on clinical superiority — it is based on awareness and habit.

Consumer research window. Implant patients research for 3–12 months before booking a consultation. They are not searching for the cheapest option — they are searching for the most trustworthy option. During that research window, they visit multiple websites, watch videos, read reviews, and build a mental shortlist. Almost no implant dentist in Louisville's suburban market has a meaningful content presence in that research phase. The window is wide open.

Fee-for-service premium. Louisville's implant market supports fee-for-service pricing at full case values of $3,500–$6,500 per single implant and $25,000–$65,000 for full-arch cases. ClearChoice's $50,000+ average creates a price ceiling reference that positions private practices favorably at $30,000–$45,000 — not as the "cheap" option, but as the local expert alternative at a rational price differential.

3B. Competitive Landscape

Pattern 1 — ClearChoice Dental Implant Center (9351 Viking Center Drive, Louisville, 40222)

- Strengths: Brand recognition, heavy TV/radio/digital advertising spend, "one location one team" messaging, professional consultation process.
- Weaknesses: Corporate pricing premium (25–40% above independent specialists). No direct dental insurance acceptance. Average $52,300 for full-arch cases. Consumer review platforms (Reddit, Google supplementary sources) average 3.2/5, with consistent complaints about financial transparency and high-pressure consultation tactics. Rotating clinical staff — patients do not always see the same doctor.
- Dr. Hedges' counter-position: *"Private care. Local doctor. Same team from your first consultation to your final crown. ClearChoice-level expertise without the ClearChoice price tag — and we work with Delta Dental."*

Pattern 2 — Innovative Oral Surgery (Multi-location specialist group)

- Strengths: Specialty credentials, multi-location convenience, 4.9–5.0 star ratings, strong referral network from Louisville GPs and periodontists.
- Weaknesses: Surgical-only scope — patients still need to return to their GP for the restorative crown and ongoing care. Complex cases are split between the surgeon and the restoring dentist, with the attendant coordination friction. Higher per-procedure fees.
- Dr. Hedges' counter-position: *"Complete implant care in one practice. Dr. Hedges handles everything — surgical placement through final crown. No referrals. No coordination between offices. Your case stays with one doctor who knows your mouth."*

Pattern 3 — Nelson & Lowy PSC (2015 Herr Lane, Suite 3 — same building as Hedges Dental)

- Profile: Periodontist practice, established 1987. Both doctors hold University of Louisville faculty appointments. Strong referral hub for complex periodontal cases in this corridor of Louisville.
- Competitive note: They are not competing for the same patient — they are the surgical backbone for patients whose periodontal disease requires specialist treatment before implant restoration.
- Strategic note: This is the most valuable referral partnership available to Dr. Hedges. They are in the same building. They see edentulous and peri-implant disease patients regularly. The restorative/implant crown work those patients need after treatment is a natural Hedges Dental referral. This relationship, properly formalized, is worth $16,500–$27,500 per month in incremental production. See Fast Cash Sprint Strategy #3 for the approach.

Pattern 4 — General dentists with "we offer implants" positioning

- The most common competitor category. Every general dentist in Louisville lists implants among their services. None of them have a content strategy, a second domain, a dedicated implant conversion page, or any material presence in the patient's pre-consultation research phase.
- Dr. Hedges' counter-position: He is already ahead of this entire category by virtue of owning toplouisvilledentist.com and having an All-on-4 page. When that site is properly built and positioned, he moves into a different category entirely.

3C. The Positioning Gap

The following positioning is unclaimed in the Louisville implant market:

> *"Louisville's private-practice implant dentist — complete care from placement to crown, in one office, with one doctor who knows your case."*

ClearChoice cannot claim this (rotating corporate staff). Oral surgeons cannot claim this (surgical only, no restorative). General dentists with passive "we offer implants" listings cannot claim this (no positioning depth). Dr. Hedges can claim it accurately and entirely.

The second unclaimed position:

> *"The implant dentist who accepts Delta Dental and charges what's fair — not what ClearChoice charges."*

In a market where the dominant full-arch brand charges $52,000 average and accepts no insurance, Dr. Hedges' Delta Dental participation and competitive pricing is not a concession — it is a weapon. It needs to be stated explicitly in implant-specific marketing contexts (not in general dentistry contexts, where "affordable" language undermines premium positioning).

3D. The Authority Statement

> *"Dr. Jeffrey Hedges is Louisville's private-practice implant dentist — a UofL-trained DMD who handles everything from implant placement to final crown in a single practice, delivering ClearChoice-level expertise with the continuity, personal attention, and straightforward pricing that only a locally-owned practice can provide."*

This statement becomes the anchor for toplouisvilledentist.com, all implant-specific content, Google Business Profile descriptions, and any paid media. It is specific, defensible, and cannot be truthfully replicated by any competitor currently operating in the Louisville market.

— Daniel Frost, CSO | Authority Systems Group™

"Strategy without sequencing is just a wish list."

Avatar: Louisville Implant Rachel — female, 48–65, Louisville-area resident researching dental implant options.

Primary Service: Dental implants (single tooth and/or full arch)

Enemy Belief being dismantled: "For something as serious as implants, I should go to a specialist or an implant center — not a regular dentist."

BELIEF TRACK — What Rachel Thinks Is True

B1 — ENEMY BELIEF

Avatar's internal monologue:

> *"I've been missing this tooth for two years. I know I need an implant. But implants are a big deal — surgery, bone, titanium. My dentist seems fine for cleanings and fillings, but for something like this? I feel like I should go to a specialist. Maybe the place on Viking Center that advertises on TV. Or an oral surgeon. A regular dentist doing implants just feels like... going to a family doctor for surgery."*

Why this belief formed: Implant marketing in most markets has been dominated by specialist groups and corporate centers. The cultural shorthand — "for something serious, see a specialist" — applies to medicine and has been imported into dentistry. ClearChoice's advertising specifically reinforces this: the imagery is clinical, the team is multiple specialists, the center is purpose-built. The framing creates the impression that anything outside this specialist model is a compromise.

What produces the transition: A piece of content that specifically addresses who is actually qualified to place implants — and what the real determinants of implant success are (training, experience, and case selection) — not the type of practice the dentist happens to run.

What Dr. Hedges specifically shows:

Content: *"Should you see a specialist for dental implants — or is your dentist qualified? A Louisville dentist answers honestly."*

B2 — PRE-FRAME

Avatar's internal monologue:

> *"Wait. So the distinction isn't really 'specialist vs. general dentist.' It's whether the dentist has the specific training and does enough cases to do it well. That's actually a more useful way to think about this."*

What produces the transition: A reframe that explains what implant training actually involves (AAID, Misch Institute, hands-on mentorship, surgical certification) and what case selection criteria look like. Rachel understands that a trained, experienced dentist is not a compromise — and that some "implant centers" use associate doctors with less experience than the private practitioner she's considering.

What Dr. Hedges specifically shows:

Content: *"What to actually ask your implant dentist before you let anyone near your jaw."* — A specific credential and experience checklist that naturally places Dr. Hedges' profile at the top.

B3 — NEW WORLDVIEW

Avatar's internal monologue:

> *"The right question isn't what kind of practice they run. The right question is: has this specific doctor placed hundreds of implants, do they handle everything in one office, and do other patients in Louisville trust them? Those are things I can actually evaluate."*

What produces the transition: Educational content that gives Rachel a completely new evaluation framework — specific, actionable criteria that position Dr. Hedges' profile favorably without ever naming competitors.

What Dr. Hedges specifically shows:

Content: *"The 5 questions Louisville residents should ask before choosing an implant dentist"* — A guide that naturally leads to Dr. Hedges' differentiators: complete care in one office, UofL training, local private practice, CareCredit financing, Delta Dental participation.

B4 — INTERNAL ALIGNMENT

Avatar's internal monologue:

> *"That makes total sense. My neighbor went to one of those corporate implant centers and paid $58,000 and said the doctor was different every time she went in. She has no idea who placed her implants. And the results were fine, but it felt like an assembly line. I don't want that. I want a doctor who knows my case from day one."*

What produces the transition: A patient story (with consent) of someone who chose a corporate implant center experience and then contrasted it against what a personalized private-practice experience delivers. Alternatively: a patient who had a fear about going to a "regular dentist" for implants and was surprised by the quality and continuity of care.

What Dr. Hedges specifically shows:

A patient story: *"I drove past that big implant center on Viking Center every time I came in. Here's why I'm glad I didn't stop."* — Real patient. Real outcome. Real contrast.

B5 — CERTAINTY

Avatar's internal monologue:

> *"Dr. Hedges is the right choice. He handles everything in one office. He's been placing implants for years. He's local. He accepts Delta Dental. He offers CareCredit. I've read three patient stories. I know what to expect at the consultation. I just need to call."*

What produces the transition: A transparent description of exactly what the consultation involves — no surprises, no sales pressure, just a clear picture of the first step.

What Dr. Hedges specifically shows:

Content: *"What happens at your free implant consultation at Hedges Dental — a step-by-step guide."* — Process transparency that eliminates the last barrier to booking.

EMOTION TRACK — What Rachel Feels Is True

E1 — DISSATISFACTION

Emotional state: Rachel has been living with a missing tooth or failing dentition for 1–3 years. She has normalized it — telling herself it's not that bad, she'll deal with it eventually, she's too busy right now. But it surfaces every time she sees a photo of herself, every time she eats something she used

to love, every time someone comments on her smile. The gap between her current dental reality and the life she wants is present every day. She's not at a crisis point — but she's past comfortable.

What Dr. Hedges specifically shows:

Content: *"If you've been putting off dealing with missing teeth — this is for you."* — An empathy-first piece that meets her where she is without judgment and introduces the concept that inaction has a real, accumulating cost.

E2 — CONTRAST

Emotional state: She has now seen — through a patient story or a before/after gallery — what someone in her exact situation looked like 12 months after getting implants. The contrast between her current state (avoiding photos, avoiding certain foods, self-conscious in social situations) and the after-state (eating anything, smiling freely, looking like herself again) is undeniable. She wants what that person has.

What Dr. Hedges specifically shows:

Patient transformation gallery on toplouisvilledentist.com. Before/after photos with brief patient narratives (consent obtained). Not just "the result was great" — but *"I hadn't eaten a steak in four years. The week after my final crown, I went to Sullivan's for my anniversary dinner."*

E3 — DESIRE

Emotional state: She is actively researching. She is comparing providers, reading reviews, watching YouTube videos about the implant process. She is building trust in specific providers. She is looking for reasons to say yes to one of them.

What Dr. Hedges specifically shows:

Google Business Profile fully optimized with 25+ reviews — specifically implant-patient reviews, not general dentistry reviews. toplouisvilledentist.com with Dr. Hedges' full credential display, patient stories, educational content, and transparent financing information.

E4 — URGENCY

Emotional state: Rachel now understands that every month she waits, her jaw is losing bone density at the site of the missing tooth. The longer she waits, the more complex (and expensive) the

eventual treatment becomes. Waiting is not neutral — it is an active choice with a compounding cost. She needs to act.

What Dr. Hedges specifically shows:

Content: *"What happens to your jaw when a missing tooth goes untreated — and why timing matters."* — Honest, educational urgency content. Not fear-mongering. Clinical reality, explained in plain language, with clear illustration of why acting sooner produces better outcomes.

E5 — RELIEF

Emotional state: The decision to call Dr. Hedges feels like resolution, not risk. She has done the research. She trusts his training and his outcomes. She knows what to expect at the consultation. She knows what the financing looks like. The anxiety has shifted from "I'm afraid to call" to "I'm ready to call." Booking feels like the first step toward getting her life back, not the first step toward an intimidating unknown.

What Dr. Hedges specifically shows:

Content: *"Free implant consultation at Hedges Dental — here's exactly what we'll discuss."* — Full process description. No surprises. CareCredit/financing prominently displayed. "No commitment to proceed — just answers."

MASTER CONTENT MAP

Stage	Belief Stage	Emotion Stage	Content Type	Core Message	CTA
B1/E1	Enemy Belief	Dissatisfaction	Educational article / GBP post	"Trained dentist vs. specialist — what actually matters"	Read full article
B2/E2	Pre-Frame	Contrast	Before/after gallery + patient story	"What my patient said after choosing private care over ClearChoice"	Read the story
B3/E3	New Worldview	Desire	Guide + checklist	"5 questions to ask your implant dentist"	Download guide / opt-in
B4/E4	Internal Alignment	Urgency	Patient case study + bone loss article	"Why she's glad she didn't wait another year"	Book consultation

B5/E5	Certainty	Relief	Consultation process page + CTA	"Free consultation — here's what we'll cover"	Schedule now

— Dr. Raymond Cross, Board | Vivienne Carr, CMO | Authority Systems Group™

Daniel Frost, Chief Strategy Officer — Authority Systems Group™

> *"The Year 1 plan below was built from a clear-eyed analysis of Dr. Hedges' market position, existing assets, and the specific belief barriers standing between his ideal patients and his front door. It is not a general dental marketing plan with his name on it. Every initiative was selected because it's the right move for this practice in this market at this moment." — Daniel Frost, CSO | Authority Systems Group™*

5A. Year 1 Strategic Overview

The Thesis: Year 1 for Hedges Dental is a positioning recalibration and pipeline activation — not a brand-new marketing build. The raw materials are already here: a trained implant dentist, a second domain, a CareCredit provider credential, community trust built over years of practice, and a same-building periodontist who is a natural referral partner. The work of Year 1 is to make all of that visible to the patients who are looking for it.

The Logic of Sequencing: Q1 focuses on full-arch implant cases — the highest-value, highest-margin service with the clearest competitive differentiation against ClearChoice. Q1 is the right starting point because it captures the maximum revenue per case while the systems are being built, and because the patient acquisition infrastructure for full-arch naturally feeds single implant case flow as a secondary channel. Q2 expands to single-tooth implants once the authority positioning is established and the content engine is producing. Q3 targets implant-retained dentures — a significant underserved population in Louisville's 55-70 demographic. Q4 focuses on the referral network (physicians, ENTs, GP-to-GP) that compounds everything built in the first three quarters.

Month 12 Picture: Dr. Hedges is doing 8–12 implant cases per month (combined single and full-arch). toplouisvilledentist.com ranks in the top 5 for "dental implants Louisville KY" organically. The Nelson & Lowy referral loop is producing 3–5 cases per month with no marketing spend. The review count on Google is 40+ with implant-specific reviews dominating. His treatment coordinator closes at 65%+. He has declined two insurance contracts he no longer needs to retain. His average production day looks materially different than it did on January 1, 2026.

5B. Quarter 1 — Foundation + Fast Cash (Months 1–3)

Focus Service: Full-Arch Implants (All-on-4, All-on-X, implant-supported full-arch restoration)

Strategic Goal: Capture dormant demand, establish authority positioning, rebuild the primary implant digital asset

Month 1 — Fast Cash + Digital Foundation

Week 1–2: Fast Cash Sprint Activation

- **Campaign 1 — Dormant Patient Reactivation**: Pull all patients from practice management software (Dentrix, Eaglesoft, or equivalent) with open implant treatment plans or documented implant diagnoses and no scheduled treatment. Send 3-touch reactivation sequence (see Section 10, Sequence 1). Christy deploys this sequence. Jeff approves the language. Timeline to first booked consult: 2 weeks.
- **Campaign 2 — CareCredit Financing Visibility Fix**: Add CareCredit callout and badge to hedgesdental.com and toplouisvilledentist.com implant pages. Update all implant page CTAs to include monthly payment framing ("As low as $89/month through CareCredit"). This change alone will measurably improve consultation request rate within 30 days.
- **Campaign 3 — Nelson & Lowy Partnership Initiation**: Dr. Hedges brings lunch to the Nelson & Lowy team. Proposes a formal cross-referral arrangement. The script for this conversation is in Section 6, Fast Cash Sprint Strategy #3.

Week 3–4: Google Business Profile Overhaul

- Add "Dental Implants" as primary service (not secondary tag)
- Add All-on-4, All-on-X, Implant-Retained Dentures, Oral Surgery as additional service categories

- Update business description: *"Dr. Jeffrey Hedges, DMD — Louisville dental implant specialist offering single-tooth implants, All-on-4, and full-arch restoration at 2015 Herr Lane. Delta Dental provider. CareCredit financing available."*
- Upload 8–12 implant-specific photos (before/after, operatory, Dr. Hedges with patient)
- Respond to all existing reviews (signals activity, improves local algorithm)
- Seed 3–5 Q&A entries with implant-specific questions and answers
- Target keyword set: "dental implants Louisville KY" · "All-on-4 Louisville" · "implant dentist Louisville" · "dental implants 40222"

Content Launch

- Publish authority article #1 on hedgesdental.com: *"Should you see a specialist for dental implants — or is your dentist qualified? A Louisville dentist answers honestly."*
- Schedule 4 social posts for Month 1 (Nextdoor + Facebook): educational implant content, no sales tone

Month 2 — toplouisvilledentist.com Rebuild + Review System

toplouisvilledentist.com Build (detailed spec in Section 9):

- Weeks 1–2: Content writing, design, copy approval
- Weeks 3–4: Build, QA, launch
- Launch milestone: All core pages live, GSC + GA4 active, consultation booking linked

Review System Activation:

- Deploy 3-step review request sequence for all new implant patients (same-day text, Day 7 follow-up, Day 30 check-in)
- Retroactively request reviews from past implant patients in the last 12 months (Christy personal outreach — 10–15 targeted contacts)
- Target: 15 new Google reviews in Month 2, specifically from implant patients

Referral Program Launch:

- Identify top 10 satisfied implant patients
- Personal outreach from Dr. Hedges (handwritten note or phone call): "If you know someone who's been putting off dealing with missing teeth, I'd love to help them the same way I helped you."
- Deploy Sequence 3 (Referral Capture) from Section 10

Month 3 — Optimization + Q2 Prep

- Review Q1 KPIs against targets (consultation bookings, show rate, close rate, review velocity)
- Document top-performing content (double down on format and topic)
- Q2 content calendar finalized: single-tooth implant focus begins
- Authority article #2 published: *"How much do dental implants really cost in Louisville — a dentist's honest answer"*
- Assess Nelson & Lowy partnership: how many referrals have been generated, what adjustments are needed

Q1 Exit Success Metrics:

- [ ] 3–8 consultations from dormant patient reactivation
- [ ] 1–3 full-arch cases started (Fast Cash Sprint revenue)
- [ ] toplouisvilledentist.com live and indexed
- [ ] GBP fully optimized and generating implant-specific impressions
- [ ] 15+ new Google reviews (implant-specific)
- [ ] Nelson & Lowy partnership formalized
- [ ] CareCredit callout live on all implant pages

5C. Quarter 2 — Expansion (Months 4–6)

Focus Service: Single-Tooth Implants + Bone Grafting

Strategic Goal: Capture the higher-volume single implant case market now that authority positioning is established

Month 4 — Single Implant Content + SEO Expansion:

Launch single-implant content track on toplouisvilledentist.com: dedicated single-implant page (600+ words, fully educational, conversion-optimized), FAQ page (schema-marked, 12 questions), authority article #3: *"Dental implant vs. dental bridge — which is actually better for your long-term health?"*

Deploy email Sequence 5 (Identity Self-Selection) for new leads entering the funnel from toplouisvilledentist.com. This sequence pre-qualifies patients before consultation, eliminating price-shoppers and unqualified candidates from the front desk workload.

Month 5 — First Patient Transformation Story Published:

Film and publish the first formal patient story — video (2–3 minutes, patient's own words) + written case study (Oliver Grant format, 4-section: before, decision, treatment, after). This becomes the most-shared piece of content on the site. Nextdoor share of the video story will generate significant inbound inquiry from the existing Neighborhood Favorite community.

Begin Nextdoor Business Page strategy: monthly educational implant posts, active response when neighbors ask about implant dentists.

Month 6 — Mid-Year Review:

- Full KPI review against 6-month targets
- Paid media evaluation: if toplouisvilledentist.com is converting at ≥3% on organic traffic, Google Ads layer is now warranted (Jordan Merritt, Paid Media — Section 12)
- Case study #1 published and distributed
- Referral network expanded: begin physician outreach (ENTs, sleep medicine, oncology practices in the 40222 corridor)

Q2 Exit Success Metrics:

- [ ] 6–10 implant cases per month (combined single + full-arch)
- [ ] toplouisvilledentist.com generating 200+ monthly sessions
- [ ] 2+ patient transformation stories published
- [ ] Physician referral outreach initiated (5+ contacts)
- [ ] Email list size: 75+ opt-in subscribers

5D. Quarter 3 — Depth + Diversification (Months 7–9)

Focus Service: Implant-Retained Dentures

Strategic Goal: Capture the denture-conversion population — high volume, underserved, strong seasonal pattern

Month 7 — Denture Patient Content Track:

Louisville's 55–70 demographic has significant denture-wearing population who are either dissatisfied with their current dentures or unaware that implant-retained alternatives exist. Dedicated content track: *"Tired of your dentures? Louisville patients are switching to implant-supported alternatives — here's why."*

Fall 2026 (September) is the second-highest conversion window of the year for implant decisions. Begin fall campaign content in August. Pre-load: educational social posts, "back to fall" scheduling callout, CareCredit year-end benefit urgency framing.

Month 8 — SEO Compounding Check:

By Month 8, toplouisvilledentist.com authority articles from Q1–Q2 are beginning to rank organically. Document and double down on the highest-performing content. Commission authority article #5 and #6 on top-ranking topic adjacencies.

Month 9 — Referral Network Health Check:

- Nelson & Lowy partnership performance review: cases generated, referral volume, relationship quality
- Physician outreach follow-up: which contacts have generated referrals, which need re-engagement
- Patient referral program results: how many satisfied implant patients have referred at least one person?

5E. Quarter 4 — Dominance + Year 2 Prep (Months 10–12)

Focus Service: Referral Network Expansion (GP-to-GP + Physician Loop)

Strategic Goal: Build the inbound referral infrastructure that runs without ad spend

Month 10 — Year-End Benefit Urgency Campaign:

"Use it or lose it" dental benefit messaging for patients with partial implant coverage. Even patients whose insurance doesn't fully cover implants respond to year-end benefit urgency. CareCredit year-end framing: *"Start your implants this year. Your CareCredit plan starts the clock — not your procedure date."*

Month 11 — Second Patient Story Published:

Full-arch transformation story. Video + written case study. This one features a patient who had been to ClearChoice (names no names, describes the experience — high price, uncertain about who would do the work) and then found Dr. Hedges. This is the highest-value piece of content for ClearChoice counter-positioning and will circulate widely in Louisville's Nextdoor communities.

Month 12 — Year-End Review + Year 2 Planning:

- Full KPI review against all targets
- Authority audit: what position has been established, what evidence exists
- Pricing review: with established authority, is there a basis for a fee increase in Year 2?
- Year 2 strategic brief: paid media scale-up, possible video content series, DSO valuation positioning if relevant

Year-End Success Targets:

KPI	Baseline (March 2026)	90-Day Target	6-Month Target	12-Month Target
Implant consultations/month	2–4	5–8	8–12	12–18
Implant cases started/month	1–3	3–5	6–10	8–12
Avg implant case value	$4,500	$4,500	$5,000	$5,500
Implant monthly production	~$9,000	$18,000–$32,000	$35,000–$55,000	$55,000–$80,000
Google review count	~5–10	25+	40+	75+
Google review rating	Unknown	4.8+	4.8+	4.9+
toplouisvilledentist.com sessions	0	Live	200+/mo	600+/mo
Email list (implant opt-ins)	0	20+	75+	200+
Referral % of implant cases	~10%	15%	25%	35%
Patient referral cases/month	0	1	3–4	5–6

— Daniel Frost, CSO | Authority Systems Group™

Your First 90 Days of Revenue Activation

Prepared by Tanya Blackwood, CRO — Authority Systems Group™

> *"Before we build anything new, let's look at what's already there. Every dental practice has revenue sitting in its existing relationships — in patients who were diagnosed and never treated, in referral sources who don't have a reason to call, in*

> *financing options that aren't being presented. The Fast Cash Sprint™ is how we go get it." — Tanya Blackwood, CRO | Authority Systems Group™*

FAST CASH STRATEGY #1

Wake the Dormant Implant-Diagnosed Patients

THE OPPORTUNITY

Your practice management software contains patients who received an implant diagnosis and never moved forward. They weren't lost — they deferred. Based on your practice's tenure and patient volume, there are conservatively 20–50 patients in this category right now. A structured 3-touch outreach to these patients is the single fastest revenue activation available, and it requires no new marketing infrastructure, no website changes, and no ad spend.

WHY THIS WORKS FOR HEDGES DENTAL

Dr. Hedges' patients trust him as their family dentist. That trust does not need to be rebuilt — it's the reason this campaign works. A reactivation message that comes from "Christy at Hedges Dental" lands completely differently than a cold message from a dental marketing service. These patients know the practice. They know the doctor. They just need a reason to re-engage on this specific treatment. The message below provides that reason without pressure.

THE APPROACH

Step 1 — Pull the list: Christy runs a report in the practice management software filtering for: patients with an implant diagnosis code (D6010, D6040, D6057, D6058, or "implant" notation) AND no scheduled implant treatment AND last contact within the past 36 months. Target list: 20–50 names.

Step 2 — Touch 1 (Text, Day 1): *"Hi [Name], this is Christy at Hedges Dental. Dr. Hedges is following up with patients who had an implant recommended at a previous visit. We're offering complimentary implant consultations this month — no charge, no commitment, just answers. Would you like to schedule 20 minutes with Dr. Hedges? Reply YES and I'll get you on the calendar."*

Step 3 — Touch 2 (Email, Day 5): Educational subject line — *"What happens to your jaw when a missing tooth goes untreated"* — soft CTA to schedule consultation at the close. Full email body is in Section 10, Sequence 1, Email 2.

Step 4 — Touch 3 (Postcard, Day 10): Before/after photo. *"Your implant consultation is complimentary. No commitment. Just answers. Call Christy at (502) [PRACTICE NUMBER] or go to toplouisvilledentist.com."*

Step 5 — Follow-up on all responses within 24 hours. Every YES gets booked within the same week.

REVENUE IMPACT ESTIMATE

Conservative: $13,500 (30 patients contacted × 10% response = 3 consults × 75% show rate × 60% close rate × 1.5 avg cases × $5,000 avg value)

Target: $37,500 (50 patients contacted × 15% response = 7–8 consults × 80% show × 65% close × 1.5 cases × $5,500 avg)

Full-arch scenario: 1 case at $35,000 changes the math entirely

Timeline: First booked consultation within 2 weeks of launch; first case start within 30 days

IMPLEMENTATION REQUIREMENTS

What ASG provides: Email copy (full body, Section 10 Sequence 1), postcard copy and design brief, text message scripts

What Dr. Hedges provides: Patient list pulled from practice management software; Christy's time to send and manage responses (approximately 3 hours for list pull and outreach)

CRM dependency: None required — Dentrix/Eaglesoft native texting and email OR personal outreach via text and email

Timeline to launch: 7 days after patient list is provided

COMPLIANCE NOTES

HIPAA: Outreach to existing patients about their own treatment plan is compliant. No PHI should be included in the message subject line. Texting should be through a HIPAA-compliant messaging platform (Weave, Podium, or Dentrix Patient Engage) rather than personal cell.

No outcome guarantees in any patient communication.

SOFIA VEGA'S TAKE

"I've run this exact campaign in three practice transitions. The list is always bigger than the dentist expects, and the response rate is always higher — because these patients are not strangers, they're people who already trust the doctor and just needed someone to re-open the door. Don't overthink the message. Simple and personal beats polished and formal every time."

FAST CASH STRATEGY #2

Lead With Financing — Retrain the Consultation Conversation

THE OPPORTUNITY

Hedges Dental is a registered CareCredit provider. This appears on CareCredit's doctor locator. It appears on neither website. At every implant consultation, cost is the primary barrier to case acceptance — not surgical fear, not time, not trust. A patient who is quoted $38,000 for a full arch and then handed a paper with the total is statistically likely to say "I need to think about it." A patient who is shown "$38,000 = $715/month for 60 months with approved credit" has a different conversation entirely.

WHY THIS WORKS FOR HEDGES DENTAL

ClearChoice leads with financing in their consultations. It's a core part of why their close rate is high despite their prices being higher — they reframe the investment as a monthly commitment rather than a lump sum. Dr. Hedges has the same CareCredit infrastructure in place and isn't using it as a conversion tool. This is not a new system to build — it's a conversation to change.

THE APPROACH

Step 1 — Website fix (this week): Add CareCredit badge and monthly payment callout to every implant page on both sites. *"Flexible financing available — full-arch cases as low as $715/month through CareCredit. Single implants from $89/month."* Link to CareCredit apply page.

Step 2 — Consultation script update: At every implant consultation, financing is presented before cost. The treatment coordinator presents: *"Before we talk about the investment, let me show you what the monthly options look like through CareCredit, because most of our patients find that changes the whole conversation."* Monthly payment is presented first. Total case cost is presented second.

Step 3 — Add Sunbit or LendingClub as secondary financing options for patients who don't qualify for CareCredit. Sunbit in particular has high approval rates for patients with imperfect credit — which is a significant portion of the full-arch candidate population.

Step 4 — Train Christy on financing objection handling: When a patient says "I need to think about it," the response is "Can I ask — is it the total investment that needs more thought, or something else? Because if it's the investment, let me show you the monthly options before you leave."

REVENUE IMPACT ESTIMATE

Conservative: +15% improvement in existing consultation close rate

If current close rate is 40% on 4 monthly consultations: 4 × 40% × $5,000 = $8,000/month → 4 × 55% × $5,000 = $11,000/month = +$3,000/month

If 1 full-arch case per month converts that previously walked away: +$35,000

Timeline: Immediate impact — first consultation after script update

IMPLEMENTATION REQUIREMENTS

What ASG provides: Website copy update, treatment coordinator script, financing objection handling guide

What Dr. Hedges provides: CareCredit merchant information for website badge; Christy's 1 hour for script review and training

CRM dependency: None — this is a conversation change, not a technology change

Timeline to launch: 3 days for website changes; script live at next consultation

COMPLIANCE NOTES

Financing content on the website must accurately represent APR, terms, and third-party provider. *"With approved credit"* qualifier required in any monthly payment claim. Do not state specific APR on the website — link to CareCredit's own rate disclosure page.

SOFIA VEGA'S TAKE

"The financing presentation is where most dentists leave the most money. They tell patients the total, watch their face fall, and then spend 20 minutes trying to overcome an objection they created by leading with the wrong number. Lead with the monthly payment. The total is a footnote."

FAST CASH STRATEGY #3

Formalize the Nelson & Lowy Referral Partnership

THE OPPORTUNITY

Nelson & Lowy PSC is in your building. They have been a periodontal practice since 1987. Both you and their doctors hold UofL faculty appointments. Their patient population includes significant numbers of people with advanced periodontal disease, bone loss, failing teeth, and edentulism — all of whom are eventual implant candidates. Right now, when Nelson & Lowy patients need restorative work after periodontal treatment, they go back to whatever general dentist they came from — not you. That's the gap.

WHY THIS WORKS FOR HEDGES DENTAL

The geography makes this unusually easy. You share a building. You share a faculty affiliation. The relationship infrastructure already exists — this is a formalization, not a cold approach. If the Nelson & Lowy team doesn't already know Dr. Hedges personally, the shared building means an introduction is as natural as it gets. Periodontal practices refer restorative cases to GPs by default — the question is just which GP gets those cases. The answer should be the GP in their building.

THE APPROACH

Step 1 — The introduction conversation (this week): Dr. Hedges brings lunch to the Nelson & Lowy team. The script: *"I've been thinking about how much we could do together for patients. When you treat someone for periodontal disease and they need implant crowns or restorations afterward, I'd love to be your first call for the restorative work — and when I have a complex surgical case that exceeds my scope, you're who I want to send them to. Seems like a natural fit given we're in the same building. What would that look like from your side?"*

Step 2 — The formal arrangement: Simple written referral agreement. Dr. Hedges handles all restorative/implant crown work for Nelson & Lowy surgical cases. For complex surgical cases (full-arch extraction, significant bone grafting) that exceed Dr. Hedges' surgical comfort, Nelson & Lowy is the referral. Mutual benefit, clearly defined.

Step 3 — Co-branded patient communication: Once formalized, create a co-branded educational sheet for Nelson & Lowy's waiting room: *"Complete implant care at 2015 Herr Lane — your comprehensive implant team, right here."*

Step 4 — Monthly case coordination call: 15 minutes per month to review pending cases and ensure nothing falls through the referral loop.

REVENUE IMPACT ESTIMATE

Conservative: 3 referral cases/month × $5,500 avg = $16,500/month incremental

Target: 5 referral cases/month × $5,500 avg = $27,500/month incremental

Full-arch scenario: 1 full-arch referral/month from Nelson & Lowy = +$35,000

Timeline: First case expected within 30–60 days of partnership formalization; full volume by Month 3

IMPLEMENTATION REQUIREMENTS

What ASG provides: Referral conversation script, co-branded waiting room sheet design brief, monthly case coordination agenda template

What Dr. Hedges provides: The conversation and the relationship; 15 minutes per month for case coordination

CRM dependency: None — referral tracking in existing patient management system

Timeline to launch: Conversation this week; agreement within 2 weeks; first case expected within 30–60 days

COMPLIANCE NOTES

Kentucky dental referral arrangements between licensed practitioners are not subject to anti-kickback statutes governing Medicare/Medicaid when both parties are fee-for-service. No financial compensation for referrals — value exchange is clinical (Dr. Hedges provides restorative; Nelson &

Lowy provides surgical where needed). No patient steering — referral is offered as a service, and patients retain the right to choose their own providers.

SOFIA VEGA'S TAKE

"The same-building referral partner is one of the most underexploited assets in all of dentistry. I had a periodontist two floors above me for six years and we passed maybe 10 patients back and forth informally. The day we sat down and made it formal — with a clear agreement and a monthly call — the number went to 8–12 per month. This is a lunch conversation away from being worth $25,000 a month."

FAST CASH SPRINT™ — 90-DAY ACTIVATION CALENDAR

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WEEK 1-2: LAUNCH PREPARATION

☐ Strategy 1: Christy pulls dormant implant-diagnosed patient list from practice
management software
☐ Strategy 2: ASG delivers website CareCredit copy and monthly payment callout
for both sites
☐ Strategy 2: Christy and Jeff review treatment coordinator financing script
☐ Strategy 3: Dr. Hedges schedules lunch with Nelson & Lowy team
☐ Client provides by Day 7: Dormant patient list (20-50 names), CareCredit
merchant badge
☐ ASG delivers by Day 7: Reactivation email copy (Sequence 1), postcard copy
brief, financing script

WEEK 3-4: CAMPAIGN LAUNCH

☐ Strategy 1: Christy sends Touch 1 text to all dormant implant-diagnosed
patients
☐ Strategy 2: CareCredit callout live on both websites — review and confirm
☐ Strategy 3: Nelson & Lowy lunch meeting — partnership conversation
☐ Strategy 1: Touch 2 email deploys Day 5 after text (see Section 10, Sequence
1, Email 2)
☐ Monitor and respond to all reactivation responses within 24 hours
☐ Book all YES responses within same week

WEEK 5-8: ACTIVE CAMPAIGN MANAGEMENT

☐ Strategy 1: Touch 3 postcard mails (Day 10 after initial text)
☐ Strategy 1: Follow up with non-responders via personal phone call from Christy
☐ Strategy 2: Training — Christy practices financing presentation before 3
consultations
☐ Strategy 3: Nelson & Lowy agreement finalized; co-branded waiting room sheet
delivered
☐ Google Business Profile optimization completed (2-3 hours, Dr. Hedges or
Christy)
☐ First 5 review requests sent to satisfied recent implant patients
☐ Weekly: Track consultation bookings and sources in KPI dashboard
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WEEK 9-12: OPTIMIZATION + LOCK IN

☐ Full reactivation campaign results review: response rate, consultations
booked, cases started
☐ Financing script performance: close rate comparison vs. pre-script baseline
☐ Nelson & Lowy: first referral cases arriving — note source and case type
☐ Review system: 15+ Google reviews targeted by Week 12
☐ toplouisvilledentist.com rebuild underway (content in draft, design approved)
☐ Build first patient case study from sprint results (Oliver Grant format)
☐ Prepare 90-day results summary for Roger review call

TOTAL ESTIMATED 90-DAY REVENUE IMPACT:
Conservative: $18,000-$30,000 (reactivation alone + financing improvement)
Target: $50,000-$110,000 (reactivation + financing improvement + first Nelson &
Lowy cases)
Full-arch scenario: $75,000-$145,000 (if 2 full-arch cases convert from
reactivation or partnership)
```

TANYA BLACKWOOD'S REVENUE CASE

These three strategies were selected over the 22 available in the Hidden Revenue Playbook because they share three qualities: they are executable within 14 days, they require no new infrastructure, and they target revenue that is already in the ecosystem.

The dormant patient list is the purest fast cash opportunity available to any established dental practice. Every patient on that list was already convinced they needed an implant — the only thing that stopped them was inertia. You're not creating demand. You're re-engaging it. The expected close rate on a properly run reactivation campaign for dental is 15–25% of contacts, not because the message is clever but because the trust already exists. One full-arch case from this list more than pays for this entire engagement.

The financing conversation change requires no ad spend, no new technology, and no patient acquisition. It is a presentation shift that will increase case acceptance on every consultation that happens from today forward. Most implant dentists table the investment without providing the monthly context. The patient's brain immediately starts doing the wrong math. Show the monthly number first. Their brain does the right math.

The Nelson & Lowy partnership is the highest long-term value play in this entire Blueprint. It is three floors down. The doctors share a faculty affiliation. The referral relationship is logical to both parties. The only thing standing between Dr. Hedges and $25,000/month in incremental production from this single partnership is one lunch conversation. That conversation should happen this week.

After the sprint: these three strategies build the foundation that everything else in this Blueprint amplifies. The reactivation confirms that marketing to implant patients works. The financing improvement confirms that case acceptance is a presentation problem, not a patient resistance problem. The Nelson & Lowy relationship confirms that referral-based growth is possible and reproducible — and opens the door to the physician referral network we build in Q2.

— Tanya Blackwood, CRO | Authority Systems Group™

"Go get the money that's already in the room before you go looking for it somewhere else."

Preston Adler, Chief Operating Officer — Authority Systems Group™

> *"Measurement is what separates a marketing investment from a marketing expense. Every number in this dashboard was chosen because it is actionable — a metric that, if it moves in the wrong direction, tells you exactly where to look and what to fix. Review this dashboard monthly. Bring the numbers to your Roger call. The data tells the story before the story becomes a problem." — Preston Adler, COO | Authority Systems Group™*

CATEGORY 1 — ACQUISITION & TOP-OF-FUNNEL

Metric	Baseline	90-Day Target	6-Month Target	12-Month Target
hedgesdental.com sessions/month	Unknown	Establish baseline	300+/mo	500+/mo
toplouisvilledentist.com sessions/month	Near zero	Live + 50+/mo	200+/mo	600+/mo
Monthly inbound inquiries (all implant sources)	2–4	6–10	10–15	15–25
Monthly inbound inquiries — full-arch specifically	0–1	2–4	4–6	6–10
Google review count (cumulative)	~5–10	25+	40+	75+

Dashboard note: Track toplouisvilledentist.com and hedgesdental.com separately in Google Analytics 4 from Day 1. The niche site's performance must be visible independently to justify the build investment and optimize content investment.

CATEGORY 2 — ENGAGEMENT & CONVERSION

Metric	Baseline	90-Day Target	6-Month Target	12-Month Target
Implant consultations booked/month	2–4	5–8	8–12	12–18
Consultation show rate	Unknown	75%+	80%+	85%+
Consultation-to-case close rate	~40%	50%+	60%+	65%+
Avg days from first contact to booked consult	Unknown	≤7 days	≤5 days	≤4 days
Lead quality score (1–10, Dr. Hedges rates weekly)	N/A	Establish baseline	7+	8+

Dashboard note: Close rate is the single most important metric in this category. The financing presentation change (Strategy #2) should produce a measurable lift in close rate by Month 2. Track close rate as a rolling 30-day average, not per-case.

CATEGORY 3 — REVENUE & VALUE

Metric	Baseline	90-Day Target	6-Month Target	12-Month Target
Implant cases started/month	1–3	3–5	6–10	8–12
Avg implant case value	~$4,500	$4,500	$5,000	$5,500
Monthly implant production	~$9,000	$18,000–$32,000	$35,000–$55,000	$55,000–$80,000
Reactivation campaign revenue (cumulative)	$0	$13,500–$37,500	—	—
Nelson & Lowy	$0	First case	$8,000–$16,500	$16,500–$27,500

referral revenue/month				

CATEGORY 4 — REPUTATION & REFERRAL

Metric	Baseline	90-Day Target	6-Month Target	12-Month Target
New Google reviews/month (velocity)	~0–1	5–8/month	5–8/month	5–8/month
Google review rating	Unknown	4.8+	4.8+	4.9+
Review requests sent/month	0	15+/month	20+/month	20+/month
Active professional referral partners	0	1 (Nelson & Lowy)	3+	6+
Patient referral cases/month	~0	1	3–4	5–6

CATEGORY 5 — CONTENT & DIGITAL

Metric	Baseline	90-Day Target	6-Month Target	12-Month Target
toplouisvilledentist.com articles published	0	5 live at launch	10	20
Email list size (implant opt-ins)	0	20+	75+	200+
Organic clicks/month (Google Search Console)	Near zero	50+/month	200+/month	500+/month
Email open rate (primary sequences)	—	Establish baseline	35%+	38%+
Patient video testimonials published	0	0	1	3+

Monthly Dashboard Review Protocol:

- Review all 25 metrics the first Monday of each month
- Flag any metric more than 20% below target for discussion in monthly Roger call

- Update KPI tracker (Airtable or Google Sheet) before the call, not during it
- Preston Adler's review team will flag anomalies and recommend adjustments

— Preston Adler, COO | Authority Systems Group™

Vivienne Carr, Chief Marketing Officer — Authority Systems Group™

> *"Dr. Hedges already has what most implant dentists in Louisville will never have: a community reputation, a second domain, and the clinical credibility to say things on camera that his competitors can't. The content strategy below turns all three of those assets into a consistent inbound stream of implant-ready patients. The goal is not to be everywhere — it's to be unmissable in the specific moment when a Louisville resident is three months into their implant research and starting to decide who to trust." — Vivienne Carr, CMO | Authority Systems Group™*

8A. Content Theme Architecture

Q1 Theme: "Implant Authority — Why Louisville Patients Choose Dr. Hedges"

Content in this theme establishes the foundational authority position. It directly addresses the B1/E1 Enemy Belief ("I should go to a specialist or implant center") and begins building the New Worldview. Every piece in Q1 drives toward toplouisvilledentist.com.

Q2 Theme: "The Implant Process — Demystified"

Content in this theme removes fear and uncertainty from the implant decision. Addresses B3/E3 — builds desire by making the process feel manageable. Targets the research-phase patient who has cleared the "who to trust" question and is now evaluating "is this right for me?"

Q3 Theme: "Real Louisville Patients, Real Results"

Social proof theme. Patient transformation stories, before/after galleries, patient-voiced video content. Targets B4/E4 — Internal Alignment and Urgency. The contrast between "before" (missing

teeth, dentures, self-consciousness) and "after" (full smile, eating freely, confidence) is the most powerful conversion content in dental.

Q4 Theme: "Implants Are More Accessible Than You Think"

Financing, insurance, candidacy, and timing content. Removes the remaining barriers for patients who are belief-aligned but cost-stalled. Targets B5/E5. This theme has seasonal power in Q4 — year-end benefit urgency and CareCredit framing.

8B. Content Calendar — Q1 Priority Pieces

Month 1 Authority Articles (toplouisvilledentist.com):

23. *"Should you see a specialist for dental implants — or is your dentist qualified? A Louisville dentist answers honestly."*

— B1 Enemy Belief dismantling. Dr. Hedges' voice. 800–1,000 words. SEO target: "dental implants Louisville KY specialist vs general dentist."

24. *"The 5 questions Louisville residents should ask before choosing an implant dentist"*

— B3 New Worldview. 900–1,100 words. Naturally positions Dr. Hedges' profile. SEO target: "best implant dentist Louisville KY."

Month 2:

25. *"How much do dental implants really cost in Louisville — a dentist's honest breakdown"*

— B3/E3. Addresses the #1 objection before the consultation. Includes CareCredit monthly framing. SEO target: "dental implant cost Louisville KY."

26. *"What happens to your jaw when a missing tooth goes untreated — and why timing matters"*

— E4 Urgency. Clinical reality content. Drives action without fear-mongering. 700–900 words.

Month 3:

27. *"Why Louisville implant patients are choosing private practices over corporate implant centers"*

— ClearChoice counter-positioning. Does not name ClearChoice. Addresses the contrast category. 800–1,000 words.

8C. Platform Strategy

Primary: toplouisvilledentist.com

All authority articles live here first. This is the asset being built for long-term SEO compounding and the destination for all implant-specific traffic. Every piece of content should drive toward this domain.

Google Business Profile

Post twice per week: 1 educational post (short-form version of the month's authority article topic) + 1 practice/patient post (non-clinical trust-building). Respond to every review within 24 hours — this signals activity to Google and to prospective patients who read reviews.

Nextdoor Business Page

Monthly educational post on implant topics relevant to Louisville neighborhoods. When a neighbor asks "does anyone know a good implant dentist?" — Christy (or Dr. Hedges) needs to have a prepared, helpful response that is visible and specific. This is the channel where the Neighborhood Favorite status becomes a lead source.

Facebook (Hedges Dental Page)

Monthly patient story share (with consent). Before/after gallery updates as cases are completed. Educational content repurposed from toplouisvilledentist.com. The Facebook audience is existing patients and community members — this is a retention and referral channel, not a new patient acquisition channel.

YouTube (Q2+)

When the first patient transformation video is ready (Month 5), publish to YouTube with full educational framing. Title: *"I finally got my teeth back — a Louisville patient's All-on-4 story."* YouTube is the second-largest search engine for healthcare information. Implant education content that ranks here compounds for years.

8D. Content Production Workflow

Responsibility allocation:

- Authority articles: ASG content team (Naomi Patel, Blog Writer) produces drafts → Dr. Hedges reviews for clinical accuracy within 48 hours → ASG publishes
- GBP posts: ASG produces 2/week in a monthly batch → Christy approves and schedules in the first week of each month
- Nextdoor posts: Christy manages with ASG-provided templates and messaging guides
- Patient stories: Dr. Hedges identifies candidates → ASG provides consent form and interview guide → ASG writes the story → Dr. Hedges and patient approve

Content approval rule: 48-hour turnaround on all content review requests. Delayed approvals delay publishing, which delays ranking, which delays leads. This is the operational commitment that Roger referenced in Section 2.

— Vivienne Carr, CMO | Authority Systems Group™

Iris Nolan, Chief Technology Officer — Authority Systems Group™

> *"hedgesdental.com serves Dr. Hedges' full patient base. toplouisvilledentist.com is something different — a single-purpose authority hub built specifically for implant patients in Louisville, fast enough to rank and focused enough to convert. While the main site continues to serve existing patients across all service types, toplouisvilledentist.com becomes the dedicated pipeline for implant cases." — Iris Nolan, CTO | Authority Systems Group™*

9A. The Strategic Case

The traffic problem: hedgesdental.com presents as a general family dentistry practice with implants listed as one of many services. An implant-researching patient who lands on the homepage sees "Professional care with a personal touch" and a broad service menu. There is no implant-specific

conversion architecture, no patient stories, no financing visibility, and no content that earns trust from someone three months into a complex research process. The site is not built to convert this patient — and it will not, regardless of how much traffic is sent to it.

The opportunity: The search terms "dental implants Louisville KY," "All-on-4 Louisville," and "dental implants 40222" represent high-intent, high-value monthly search volume with relatively low competitive content quality. The top-ranking results for these terms are dominated by directory pages (Zocdoc, Healthgrades), ClearChoice's highly-optimized location page, and generic GP listings. A purpose-built, content-rich, technically clean domain focused exclusively on implants can compete for these terms within 90–120 days of launch.

The domain asset: toplouisvilledentist.com is already owned by Dr. Hedges. There is an existing All-on-4 page with some content. The domain history is clean. The strategic instinct behind acquiring it is correct. The build investment required to turn it into a competitive implant authority site is estimated at 30–45 days of focused development.

9B. Technical Specifications

Platform: WordPress with Astra theme (lightweight, mobile-first, no page builder bloat)

Target load time: Under 2 seconds (Google PageSpeed score 90+)

Hosting: WP Engine or Kinsta — US-based server, SSL from Day 1, HTTP auto-redirect to HTTPS

Tracking: Google Analytics 4 + Google Search Console + Google Tag Manager installed before launch

Conversion tracking: Form submissions, click-to-call events, CareCredit link clicks — all tracked in GA4

Mobile standard: All CTAs (click-to-call, consultation booking form, CareCredit link) functional on mobile without zooming

9C. Site Architecture at Launch

```
toplouisvilledentist.com
├── Home (primary conversion page)
│   ├── H1: "Louisville's Implant Dentist — Complete Care from Placement to
Crown"
```

```
│   ├── Authority statement (pull from Section 3D)
│   ├── Before/after gallery (minimum 3 cases at launch — with consent)
│   ├── Dr. Hedges credentials block: DMD, UofL faculty, implant training, years
in practice
│   ├── CareCredit badge + monthly payment callout
│   ├── Google reviews widget (auto-updates)
│   ├── "Why Dr. Hedges vs. ClearChoice or a Specialist" section
│   └── Free consultation CTA (booking form or Calendly link)
│
├── About Dr. Hedges
│   ├── Professional background — implant training, UofL faculty, years in
practice
│   ├── "Why I do implants" — personal voice from Dr. Hedges (2-3 paragraphs,
approachable)
│   ├── Credentials and memberships (AAID, AGD, Kentucky Dental Association)
│   ├── Practice location and team (Christy feature — builds front-desk trust)
│   └── Photo: Dr. Hedges in the operatory + headshot
│
├── Full-Arch Implants (All-on-4 / All-on-X)
│   ├── 700-900 words — educational, process-oriented, fear-reducing
│   ├── Candidacy criteria, treatment timeline, what the day of surgery looks
like
│   ├── CareCredit monthly payment callout ("from $715/month with approved
credit")
│   ├── FAQ (5 questions with full answers, schema-marked)
│   └── CTA: Free full-arch consultation
│
├── Single-Tooth Implants
│   ├── 700-900 words — same structure
│   ├── "Implant vs. bridge — which is right for you?" section
│   ├── CareCredit callout ("from $89/month with approved credit")
│   ├── FAQ (5 questions)
│   └── CTA: Free consultation
│
├── Implant-Retained Dentures
│   ├── 600-800 words — focused on denture-wearers who don't know this option
exists
│   ├── Comparison: traditional dentures vs. implant-retained
│   └── CTA: Free consultation
│
├── Why Dr. Hedges? (Positioning page)
│   ├── Direct answer to "GP vs. specialist vs. corporate implant center"
│   ├── "Complete care in one office" differentiator
│   ├── Private practice vs. corporate — personal attention framing
│   ├── Delta Dental participation + CareCredit
│   └── Location: 2015 Herr Lane — same building as Nelson & Lowy for complex
surgical support
│
├── Patient Stories (social proof hub)
│   ├── Launch with 2-3 stories (text + photo, patient consent obtained)
│   ├── Video story added Month 5 (Owen Drake, YouTube content — Section 12)
│   └── Schema-marked for rich results
│
├── Free Consultation
│   ├── "Here's what we'll cover" — full process description (removes friction)
│   ├── Booking form (name, phone, email, "which tooth/teeth concern you?")
│   ├── Click-to-call prominent
│   └── CareCredit + Delta Dental callout
```

```
│
├── Blog / Implant Education (5 articles at launch — Q1 priority pieces from
Section 8C)
│
├── FAQ (standalone page)
│   ├── 12-15 questions with full answers
│   ├── Schema markup for featured snippets
│   └── Topics: cost, pain, candidacy, timeline, financing, GP vs. specialist
│
└── Lead Magnet Landing Page
    ├── "The Louisville Implant Guide: 7 Things to Know Before You Commit"
    ├── Opt-in form: name + email → triggers Welcome Sequence (Section 10,
Sequence 2)
    └── PDF delivered automatically; subscriber enters email nurture
```

Pages NOT at launch (add Q2+): Video library, testimonials standalone page, Spanish-language pages

9D. Local SEO Strategy

Primary keywords (highest intent, targeted from Day 1):

- "dental implants Louisville KY"
- "All-on-4 Louisville"
- "implant dentist Louisville KY"
- "dental implants 40222"
- "tooth replacement Louisville"

Secondary / long-tail keywords:

- "how much do dental implants cost in Louisville"
- "dental implants vs dentures Louisville"
- "All-on-4 vs All-on-X Louisville"
- "implant-retained dentures Louisville KY"
- "dental implant consultation Louisville"
- "ClearChoice alternative Louisville"

On-page checklist — every page at launch:

- [ ] Primary keyword in H1
- [ ] Primary keyword in first 100 words of body copy
- [ ] Meta title: Primary keyword + location (under 60 characters)
- [ ] Meta description: CTA-oriented, includes keyword + location (under 155 characters)

- [ ] Image alt text: Descriptive with keyword where natural
- [ ] Internal links: Each service page links to 2+ related pages
- [ ] Schema: LocalBusiness on home; FAQ schema on FAQ/service pages; Article schema on blog

Google Business Profile sync:

- GBP linked to toplouisvilledentist.com as secondary web presence (or as primary implant URL in services)
- GBP "From the business" description updated to match toplouisvilledentist.com authority statement
- Google Posts live within 7 days of site launch

9E. Performance Milestones

Milestone	Target Date	Success Metric
Site live	Day 45 from engagement start	All core pages published, GSC + GA4 active
First GSC impressions	Day 60	GSC showing ≥1 impression for target keyword
First consultation from site	Day 75	Form submission or click-to-call tracked in GA4
50+ monthly sessions	Month 3	GA4 sessions report
Top-20 ranking "dental implants Louisville KY"	Month 4	GSC position report
200+ monthly sessions	Month 6	GA4 report
Top-10 ranking for primary keyword	Month 5–6	GSC position report
Google 3-Pack appearance for implant terms	Month 6–9	Manual SERP check from Louisville IP

9F. Compliance Requirements

All pages include site-wide footer disclaimer: *"This website is for informational purposes only and does not constitute dental advice. Outcomes vary. Consult with Dr. Hedges to determine if dental implants are right for your situation."*

Outcome language: "typically," "in most cases," "results vary" — no guarantees.

Before/after photos: signed HIPAA-compliant patient consent required before any photo is published.

Award and recognition claims: "As recognized by [organization]" — no standalone superlatives without attribution.

Kentucky State Dental Board advertising rules: "Implant specialist" language requires specific credentials — Dr. Hedges should not be described as a "dental implant specialist" without confirming this aligns with Kentucky dental board advertising definitions. "Implant dentist" and "experienced in dental implants" are safe alternatives.

CareCredit: "With approved credit" qualifier required on all monthly payment references.

— Iris Nolan, CTO | Authority Systems Group™

Vivienne Carr, Chief Marketing Officer — Authority Systems Group™

> *"Every sequence in this section is deployment-ready. Subject lines are written. Body copy is complete. Christy should be able to take Sequence 1 and have it in patients' inboxes this week. Every email has one job — move the reader one step forward. Nothing more." — Vivienne Carr, CMO | Authority Systems Group™*

SEQUENCE 1: DORMANT IMPLANT-DIAGNOSED PATIENT REACTIVATION

Trigger: Patient has open implant treatment plan or documented implant diagnosis with no scheduled treatment

Stage flow: E1 → E2 → E3 (Emotion-first — these patients already have the belief foundation)

Deployment: Christy manages from front desk. Texts sent from practice phone or Weave/Podium. Emails from practice email.

EMAIL 1 — THE PERSONAL RECONNECT

Day 1 | Stage: E1 — Surface the gap between current state and implant-free life

Text message sent same day:

> *"Hi [First Name], this is Christy at Hedges Dental. Dr. Hedges asked me to reach out — your records show you were diagnosed with a missing tooth that's a great candidate for an implant. We're offering complimentary implant consultations this month. No charge, no commitment — just time with Dr. Hedges to answer your questions. Would you like to schedule? Reply YES and I'll get you set up."*

EMAIL 2 — THE COST OF WAITING

Day 5 | Stage: E1/E4 — Dissatisfaction surfaced; urgency introduced gently

Subject: What happens when a missing tooth goes untreated (this surprised me)

[First Name],

I wanted to share something with you — not as a sales message, but as information I think you deserve to have.

When a tooth is missing, the jawbone underneath it doesn't stay put. It starts to resorb — to shrink and flatten over time, because there's no longer a root stimulating it with the pressure of biting and chewing. This process starts within the first few months and continues for years.

Why does this matter for you?

Because the longer a missing tooth goes without an implant, the more bone you lose. And bone is what implants are anchored to. A patient who acts within 12–18 months of losing a tooth often needs a straightforward implant placement. The same patient who waits 3–4 years may need bone grafting first — an additional procedure that adds cost and healing time.

I'm not telling you this to pressure you. I'm telling you because a lot of patients don't know it, and I'd rather you have the information now than discover it later when the options have narrowed.

If you've been thinking about it and just haven't gotten around to scheduling — this month might be a good time to have the conversation. The consultation is complimentary and completely without obligation.

You can call the office at [PHONE], or if it's easier, just reply to this email with a day and time that works.

—Christy and Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

[PHONE] | toplouisvilledentist.com

To be removed from future communications, reply "REMOVE" to this email.

EMAIL 3 — THE BEFORE/AFTER

Day 8 | Stage: E2 — Contrast: what their life looks like after the implant

Subject: "I hadn't eaten a steak in four years"

[First Name],

One of Dr. Hedges' patients — a 58-year-old Louisville man who had been missing a back molar for almost four years — said something to us at his final appointment that stuck with me.

He said: *"I hadn't eaten a steak in four years. The week after you put the crown on, I went to Sullivan's for my anniversary dinner and ordered the ribeye. My wife cried."*

He wasn't dramatically impaired before the implant. He'd adapted. He chewed on the other side. He avoided certain foods. He told himself it wasn't a big deal.

But that adaptation has a cost — not just to the bone, but to the quality of the life you're living with missing teeth. Most people don't realize how much they've adjusted until they don't have to anymore.

Your consultation with Dr. Hedges is complimentary. You don't have to commit to anything. You'll know within 20 minutes what your options are, what the process looks like, and what it would cost — including the monthly financing options through CareCredit that most of our patients find make the whole thing very manageable.

Call us at [PHONE] or book directly at toplouisvilledentist.com.

—Christy and Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To be removed from future communications, reply "REMOVE" to this email.

EMAIL 4 — THE REFERRAL BRIDGE

Day 12 | Stage: E3/E4 — Desire for themselves or someone they know

Subject: If you're not ready — would you share this with someone who is?

[First Name],

I understand that timing isn't always right. If now isn't the moment for you, no pressure — we'll be here when it is.

But I wanted to ask you one thing before I leave this conversation for a while:

Is there someone you know who's dealing with missing teeth and hasn't done anything about it? A spouse, a sibling, a close friend who you've noticed avoiding photos or eating carefully?

If so, feel free to forward this email with a note. Tell them you found a dentist in Louisville who does free implant consultations and has helped a lot of people get their smile back. That's it.

We take good care of the people our patients send us.

[PHONE] | toplouisvilledentist.com/free-consultation

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To be removed from future communications, reply "REMOVE" to this email.

EMAIL 5 — THE FINAL OPEN DOOR

Day 18 | Stage: E5 — Relief available; door stays open

Subject: Last note from me on this

[First Name],

I'm going to keep this short.

If you've been thinking about the implant conversation and haven't found the right moment — I just want you to know the door is open whenever you're ready. Dr. Hedges does complimentary consultations because he believes you should have the information before you make any decision. No pressure. No obligation. Just answers.

When the time is right: [PHONE] or toplouisvilledentist.com.

We'll be here.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To be removed from future communications, reply "REMOVE" to this email.

SEQUENCE 2: NEW LEAD WELCOME SEQUENCE

Trigger: New opt-in from toplouisvilledentist.com lead magnet or consultation request form

Stage flow: B1/E1 → B2/E2 → B3/E3 → B4/E4 → B5/E5

Length: 5 emails over 7 days

EMAIL 1 — THE PATTERN INTERRUPT WELCOME

Day 0 (immediate) | Stage: B1/E1 — Recognition, not promotion

Subject: Before you make any decisions about implants — read this

[First Name],

You just downloaded the Louisville Implant Guide. Before you open it, I want to tell you something that most dental websites won't.

Most people who research dental implants for six months, compare three dentists, and read every article they can find — still end up in the wrong consultation. Not because they didn't do their homework. Because they didn't know what they were looking for.

The guide you have covers the clinical stuff: the process, the timeline, the cost range, the candidacy factors. But over the next few days, I'm also going to send you a few short notes about the things nobody puts in the brochure — including the one question almost nobody asks that tells you everything about whether a dentist is the right person to trust with this.

Start with the guide. I'll be back in 24 hours.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

(502) [PHONE] | toplouisvilledentist.com

To unsubscribe, reply "UNSUBSCRIBE" to this email.

EMAIL 2 — THE ENEMY BELIEF EMAIL

Day 2 | Stage: B1→B2 — Name and dismantle "I should see a specialist"

Subject: The belief most implant patients have that costs them (I had to say something)

[First Name],

There's a belief almost every implant patient starts with, and it's worth naming directly:

"For something as serious as implants — surgery, bone, a significant investment — I should go to a specialist or a dedicated implant center. My regular dentist is fine for cleanings, but this is different."

That belief is understandable. It formed from a combination of how dentistry is marketed (specialist groups spend heavily to reinforce it) and a general cultural shorthand — when something matters, see a specialist.

Here's the actual truth about implant outcomes:

The research consistently shows that implant success rates are not determined by the type of practice the dentist runs. They're determined by three things: the specific training the dentist has completed, the number of cases they've done, and the quality of the treatment planning for your specific bone and bite situation.

A dentist who has completed hands-on implant surgical training and places 8–15 implants per month has materially better outcomes than a dentist who completed the same training 12 years ago and places 2 per month — regardless of whether either of them works in a "specialist" group or a private practice.

What the corporate implant centers don't tell you is that their "team of specialists" often includes associate doctors on rotating schedules. The doctor who places your implant is frequently not the doctor who did your consultation. The doctor who delivers your final crown may be different from either.

At Hedges Dental: the doctor you meet on your first visit is the doctor who handles your case. Same face. Same person who reviewed your X-rays. Same person who knows your jaw.

Tomorrow I'll send you the five questions that actually determine whether you've found the right implant dentist — regardless of what kind of practice they run.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To unsubscribe, reply "UNSUBSCRIBE" to this email.

EMAIL 3 — THE NEW WORLDVIEW

Day 3 | Stage: B3/E3 — New evaluation framework + desire

Subject: The 5 questions that actually tell you if a dentist is the right choice for implants

[First Name],

Yesterday I explained why the "specialist vs. general dentist" question is the wrong question. Today I want to give you the right questions.

Before you agree to let any dentist place an implant, ask these five:

1. What implant training have you completed — and when?

Look for: Misch Institute, AAID, hands-on surgical curriculum. "I took a weekend course in 2019" is different from "I've completed surgical mentorship and do this regularly." Either can be valid — the question surfaces the difference.

2. How many implants do you place per month?

Look for: Volume sufficient to maintain skill. A dentist placing 2–3 implants per month is a different conversation from one placing 12–18. There's no minimum — but you deserve to know the answer.

3. Who handles my case from start to finish?

Look for: One doctor, beginning to end. If the answer is "our surgical specialist places it and then you return to your regular dentist," that's a coordination process — and you should understand it clearly before agreeing.

4. What financing options do you offer?

Look for: CareCredit, Sunbit, or in-house options. Full-arch cases ranging from $25,000–$50,000 become $500–$900/month conversations with the right financing. A practice that can't answer this question isn't making it easy for you to say yes.

5. Can I speak with a patient who had a similar case?

Look for: A yes. A dentist who is proud of their outcomes doesn't hesitate to connect you with a patient who has given consent to speak with prospective patients.

My answers: AAID-trained and currently placing [X] implants per month. Same doctor from consultation through final crown. CareCredit and Sunbit financing, cases starting from $89/month for single implants and $715/month for full arch. Patient references available by request.

You can book a free consultation at toplouisvilledentist.com/free-consultation — or call Christy directly at [PHONE].

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To unsubscribe, reply "UNSUBSCRIBE" to this email.

EMAIL 4 — THE INTERNAL ALIGNMENT EMAIL

Day 5 | Stage: B4/E4 — Cost of waiting; past failures explained

Subject: She waited 3 years. Here's what it cost her — and I don't mean money.

[First Name],

A patient of mine — I'll call her Carol — came to see me about a full upper arch in the spring of 2025. She had been missing most of her upper teeth for three years. She'd been wearing a partial denture. She'd adapted.

She had done her research. She'd been to ClearChoice first — the consultation was professional, the center was impressive, and the quote was $58,000. She couldn't afford it. She left without scheduling.

She spent the next 18 months convincing herself the denture was fine.

When she came to me, the news was harder than it needed to be: three years of bone loss meant she needed significant grafting in two sites before I could place implants. That added a procedure, four additional months of healing, and $4,200 in grafting costs that she wouldn't have needed if she'd acted earlier.

The implants are beautiful. She's thrilled. But she paid more and waited longer than she had to — not because implants were out of reach, but because the first quote she got was so far from her budget that she stopped looking for an alternative.

When she found out that her case at Hedges Dental — with the full-arch implants she needed — was $34,500, she cried in the chair.

If you've been researching implants and a corporate center quote made you feel like this wasn't possible — please come in for a free consultation before you decide that. ClearChoice-level outcomes at a private practice price is not a marketing line. It's a math problem that many Louisville patients have solved.

toplouisvilledentist.com/free-consultation or call Christy at [PHONE].

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

*Patient details anonymized with consent. Results vary. *To unsubscribe, reply "UNSUBSCRIBE."*

EMAIL 5 — THE CONVERGENCE EMAIL

Day 7 | Stage: B5/E5 — Certainty + Relief → consultation booking

Subject: What happens at your free consultation at Hedges Dental

[First Name],

Over the past week, you've gotten the guide, the research framework, and the honest story behind one patient's journey. If any of it resonated — here's what the next step actually looks like.

The free implant consultation at Hedges Dental:

- It's 20–30 minutes with Dr. Hedges, not a front-desk intake.
- We'll review your existing X-rays or take a quick set if needed.
- Dr. Hedges will tell you specifically what he sees, whether you're a candidate, what the treatment would involve, and what it would cost — with monthly financing options included.
- You will leave with a clear answer, a written treatment plan if you want one, and zero pressure to schedule anything.

That's it. No sales presentation. No "limited time offer." Just information from a doctor who will treat you the same whether you book the next day or six months from now.

To schedule: toplouisvilledentist.com/free-consultation

To call: [PHONE] — ask for Christy

We see patients Monday through Friday and have early-morning appointments available most weeks.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To unsubscribe, reply "UNSUBSCRIBE" to this email.

SEQUENCE 3: REFERRAL CAPTURE (FROM SATISFIED IMPLANT PATIENTS)

Trigger: Final crown delivery + positive patient feedback (thumbs up, verbal "thank you," 5-star review)

Stage flow: E5 reinforcement → referral ask

Length: 3 emails over 12 days

EMAIL 1 — THE OUTCOME CELEBRATION

Day 1 (2–3 days after final crown delivery) | Stage: E5 reinforcement

Subject: What you just did

[First Name],

I wanted to take a moment to say something that doesn't always get said:

What you did was not easy.

You made a significant decision about your health, your appearance, and your finances. You showed up. You healed. And now you have a result that is going to change the way you experience the next 20 years.

The implant you have will last as long as you maintain it. It functions and feels like a natural tooth. You will stop thinking about it within a week.

I'm proud of the outcome. Thank you for trusting us with it.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

No action required on this email — just a note of thanks.

EMAIL 2 — THE REFERRAL EDUCATION

Day 5 | Stage: E3/E4 — Desire for their referral; structured ask

Subject: I want to ask you something directly

[First Name],

I want to ask you something — but first let me explain why.

Most people who end up with missing teeth and don't do anything about it aren't afraid of the procedure. They're afraid of two things: the cost and not knowing who to trust. Those are solvable problems. The cost is solvable with CareCredit financing — most cases work out to less than a car payment per month. The trust problem is solvable with a recommendation from someone who has already been through it.

That's where you come in.

Is there someone in your life — a family member, a close friend, a neighbor on Nextdoor — who you've noticed avoiding photos, avoiding certain foods, or has mentioned missing teeth but hasn't done anything about it?

If so, here's what I'd ask you to do: forward this email with a note that says something like, *"This is my dentist. He does free consultations and he's the reason I got my smile back. Call them."* That's it. One message. You just solved both of their problems.

We take care of the people our patients send us.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

[PHONE] | toplouisvilledentist.com

To unsubscribe, reply "UNSUBSCRIBE."

EMAIL 3 — THE EASY ASK

Day 12 | Stage: E4 — Make it effortless

Subject: The easiest thing you could do for someone you know

[First Name],

One more quick note.

If you know someone who's been putting off dealing with missing teeth, here's the easiest possible way to help them:

Copy and send this text:

"My dentist does free implant consultations in Louisville. He's the one who fixed my tooth. If you've been thinking about it, call Christy at [PHONE] or go to toplouisvilledentist.com. No pressure — just worth a conversation."

That's all. One text message. We'll handle everything from there.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

To unsubscribe, reply "UNSUBSCRIBE."

SEQUENCE 4: PROFESSIONAL REFERRAL PARTNER OUTREACH

Trigger: Identifying a new professional referral partner candidate (GP, physician, ENT, Nelson & Lowy)

Stage flow: Professional-to-professional, value-first

Length: 3 emails over 21 days

EMAIL 1 — THE VALUE-FIRST INTRODUCTION

Day 1 | Professional tone — lead with what you provide for their patients, not what you're looking for

Subject: Dental implant care for your patients — Dr. Jeff Hedges, Hedges Dental

Dear Dr. [Name],

I wanted to introduce myself — I'm Dr. Jeff Hedges, DMD, at Hedges Dental on Herr Lane in Louisville. I'm reaching out because I suspect we serve some of the same patients, and I wanted to make sure you have my information.

I've invested significantly in implant training over the past several years and handle full-arch cases (All-on-4, All-on-X) as well as single-tooth implants and bone grafting — all in our office, start to finish. For your patients who are missing teeth or living with failing dentition, a straightforward referral to us means they get complete implant care with the same doctor from consultation through final crown.

We accept Delta Dental and offer CareCredit financing, which removes the two most common barriers your patients face when implants are recommended.

If you have patients who are implant candidates and you'd like a reliable referral destination, I'd be glad to connect. I'm also available to see patients quickly — typically within a week for implant consultations.

Would a brief phone call or a visit to our office make sense? I'm happy to bring lunch to your team and walk through the cases we're able to handle.

Respectfully,

Dr. Jeffrey Hedges, DMD

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

[PHONE] | toplouisvilledentist.com

EMAIL 2 — THE FOLLOW-UP WITH SPECIFICS

Day 10 | Adds clinical detail and makes the referral process concrete

Subject: Following up — what a referral to Hedges Dental looks like

Dr. [Name],

Following up on my note from last week. I wanted to make the referral process concrete in case that's useful.

When you refer a patient to us for an implant evaluation:

- We see them within 5–7 days in most cases
- We send you a brief treatment plan summary and CBCT review within 48 hours of their consultation
- For cases that involve complex surgical components outside our scope, we coordinate with Nelson & Lowy PSC (same building) — your patient stays in our care network
- For restorative cases that originate with another oral surgeon or periodontist, we're glad to handle the crown and final restoration

The patient doesn't fall through the cracks. You stay informed. That's the commitment.

If you'd like to see the practice and meet our team, I'd be glad to set that up at your convenience.

Dr. Jeffrey Hedges

[PHONE] | jeffhedges@hedgesdental.com

EMAIL 3 — THE DIRECT ASK

Day 21 | Short, direct, leaves the door open

Subject: One question

Dr. [Name],

I've reached out twice and I want to respect your time, so I'll make this brief.

If you have a patient who's a candidate for implants and you're not sure where to send them — send them to me. I'll take good care of them and I'll keep you informed.

If that's not something you need right now, no problem. My information is here whenever it is.

Dr. Jeff Hedges | [PHONE] | toplouisvilledentist.com

SEQUENCE 5: IDENTITY SELF-SELECTION SEQUENCE (PRE-CONSULTATION)

Trigger: New lead who has opted in or requested a consultation — before the consultation call

Stage flow: Attention → Relevance → Interpretation → Possibility → Identity Commitment

Purpose: Pre-qualify implant patients and filter out price-shoppers, unqualified candidates, and misaligned expectations before they reach the front desk

EMAIL 1 — ATTENTION → RELEVANCE

Subject: Before your consultation — something worth reading

Not every dental implant patient is the right fit for our practice.

That's an unusual thing to say on the way to a consultation. Most dental offices want to fill their schedule. I want to do something different — I want to make sure the patients I work with are people I can genuinely help, in the way they actually want to be helped.

Implant dentistry is not a transaction. It involves surgery, healing, multiple appointments, and a multi-year commitment to maintenance. The patients who do best are the ones who come into the process with clear expectations, a genuine commitment to their oral health, and an understanding that the right outcome takes time.

If that describes how you approach healthcare decisions — I'd like to show you exactly what we do and why our patients get the results they get.

If you're primarily looking for the fastest or cheapest option, I'm probably not the right fit — and I'd rather tell you that now than waste your time.

Over the next few days I'll send you a few short notes that will help you understand what to expect from our practice and from the implant process. Read them at your own pace.

—Dr. Jeff Hedges

To unsubscribe, reply "UNSUBSCRIBE."

EMAIL 2 — RELEVANCE → INTERPRETATION: TWO KINDS OF IMPLANT PATIENTS

Subject: Two kinds of patients — which one are you?

In my experience, implant patients sort themselves into two categories fairly quickly.

The first is the patient who has been researching implants for months. They've read about the process. They've seen the before/after photos. They've done the math on the cost and financing. They come to the consultation with questions about candidacy, healing timelines, and what the final result looks like. They're committed to the outcome — they just want to confirm they've found the right doctor.

The second is the patient who is in the early information-gathering phase. They've seen an ad, or a friend mentioned something, or they called three offices this week to get quotes. They're not sure yet whether they want implants or veneers or "whatever is cheapest." They'll decide based on who gives them the best price.

Both patients are completely reasonable human beings. I'm genuinely not the right fit for the second one — not because there's anything wrong with their approach, but because what they're looking for and what we provide are different things.

The first patient gets our full attention, a complete treatment plan, and a doctor who knows their case from Day 1.

If you're the first patient — I'll see you at your consultation.

—Dr. Jeff Hedges

To unsubscribe, reply "UNSUBSCRIBE."

EMAIL 3 — INTERPRETATION: WHY OUTCOMES DIFFER

Subject: Why two patients with the same problem get different results

People sometimes ask why implant outcomes vary so much between practices.

The clinical answer involves bone quality, implant brand, surgical technique, and healing. But those factors are less variable than most patients assume — modern implant systems are highly standardized, and success rates across trained providers are consistently in the 95%+ range.

The outcome differences that actually matter to patients aren't clinical — they're relational.

The patients who come out of the process saying "I'd do it again without hesitation" are almost universally the ones who knew what to expect at every stage, felt like their doctor knew their case individually, and had a practice they could call when something felt unfamiliar. The healing process involves weeks of adjustment. Questions come up. Small anxieties surface. The practices where

patients feel held through that process — and not just managed through it — consistently produce different experiences.

That's what our model is built for. One doctor. Continuous relationship. You can reach Christy by text if something comes up at 9pm on a Tuesday, and she'll make sure I know about it.

—Dr. Jeff Hedges

To unsubscribe, reply "UNSUBSCRIBE."

EMAIL 4 — POSSIBILITY: WHAT THE RIGHT PATH LOOKS LIKE

Subject: What the patients who do well have in common

The patients who come out of implant treatment the most satisfied tend to share a few characteristics.

They came to the first consultation having already done some reading — not to impress anyone, just because they took the decision seriously. They asked questions about bone health and candidacy rather than leading with cost. They were honest about their medical history, their medications, and their habits. They followed the post-surgical care instructions because they understood why each one mattered, not because they were told to.

They also tended to be the patients who felt comfortable asking questions throughout the process — at the consultation, during healing, and afterward. Not anxious questions, but informed ones.

Those patients get excellent outcomes. Not because they're special. Because the relationship works the way it's supposed to work when both people are in it seriously.

If that's how you approach your health care decisions, I think we'll work well together.

—Dr. Jeff Hedges

To unsubscribe, reply "UNSUBSCRIBE."

EMAIL 5 — IDENTITY COMMITMENT

Subject: Before I see you

One last thought before your consultation.

The practices that produce the best implant outcomes — not just technically, but experientially — operate on the same basic principle: the right outcome takes the right relationship.

I am selective about which cases I take. Not because I need to be selective for capacity reasons. Because a patient who isn't ready for the commitment involved in implant treatment — the healing, the maintenance, the investment — won't have the outcome they're hoping for. And that doesn't serve either of us.

If you've read these notes and felt like I was describing the way you think about healthcare — then your consultation is going to be a straightforward conversation between two people who are both serious about the right outcome.

If something in these notes made you feel like this is more than you wanted to get into — that's useful information too, and there's no embarrassment in it.

I'll see you at your consultation. Christy will confirm the time.

—Dr. Jeff Hedges

Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222

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Vivienne Carr, CMO (Lead Magnet) + Tanya Blackwood, CRO (Referral Program)

Authority Systems Group™

11A. THE LEAD MAGNET

Title: *The Louisville Implant Guide: 7 Things to Know Before You Commit to Dental Implants*

Strategic Role: This guide captures implant-researching patients at B1/E1 — before they've decided on a provider, while they're still in full research mode. It enters them into Sequence 2 (Welcome Sequence) and begins the Belief-to-Buy journey in document form. A patient who reads this guide before their consultation arrives significantly more informed and more pre-sold than a patient coming in cold.

Format: PDF, 12–15 pages, branded for toplouisvilledentist.com. Dr. Hedges' headshot on page 2. CareCredit badge and "free consultation" CTA on pages 7, 11, and final page.

Distribution:

- Primary: toplouisvilledentist.com opt-in form (triggers Sequence 2 automatically)
- Secondary: Nelson & Lowy waiting room (co-branded version: "From the implant team at 2015 Herr Lane")
- Tertiary: Sequence 1, Email 3 attachment for dormant patients

TABLE OF CONTENTS — Full Guide Structure

Section	Stage Pair	Title
1	B1/E1	Why most people wait too long — and what it actually costs
2	B2/E2	The specialist vs. general dentist question, answered honestly
3	B3/E3	Am I a candidate? A plain-language guide to eligibility
4	B3/E3	The implant process, step by step — what to expect from day 1
5	B4/E4	Real Louisville patients: before and after (with permission)
6	B4/E4	The real cost of dental implants in Louisville — and why timing matters
7	B5/E5	Financing options: how a $35,000 case becomes $650/month
8	B5/E5	5 questions to ask any implant dentist before you agree to

		treatment
9	B5/E5	Why Dr. Hedges — and what makes Hedges Dental different
10	→ BUY	Your free consultation — here's what to expect and how to schedule

Compliance requirements on every page footer: "This guide is for informational purposes only and does not constitute dental advice. Outcomes vary. Results presented with patient consent. Consult Dr. Hedges to determine if dental implants are appropriate for your situation. Hedges Dental | 2015 Herr Lane, Suite D | Louisville, KY 40222 | [PHONE]."

11B. THE REFERRAL PROGRAM

Tanya Blackwood, CRO — Authority Systems Group™

Program Name: The Herr Lane Circle — Dr. Hedges' Patient Referral Program

Core Mechanism: Three referral channels operating simultaneously — Patient-to-Patient, Professional (Nelson & Lowy and area GPs), and Physician Loop (ENTs, sleep medicine, oncology). Each channel has its own activation protocol and tracking mechanism.

CHANNEL 1 — Patient-to-Patient

Who: Satisfied implant patients (single or full-arch) who completed treatment within the past 24 months

The Ask Moment: Immediately after final crown delivery — when E5 (Relief) is at its highest. Sequence 3 automates this.

The Incentive: No financial incentive (HIPAA and state dental board compliance). Instead: a personal thank-you from Dr. Hedges by phone when a referral case starts. Recognition in the practice's annual patient communication. This is sufficient — satisfied implant patients don't refer for rewards. They refer because the experience was meaningful enough to share.

The Tool: A physical card given to each satisfied patient at final appointment: *"Know someone who might benefit from what you just experienced? Give them this."* Card front: Dr. Hedges photo + practice name + "Free implant consultation." Card back: toplouisvilledentist.com + Christy's direct number.

Target: 1 referral per quarter from every satisfied implant patient. At 12 completed implant cases per year, this generates 12 warm referrals per year — approximately 4–6 of which convert to new cases.

CHANNEL 2 — Professional Referral (Nelson & Lowy + Area GPs)

Who: Nelson & Lowy PSC (formalized per Fast Cash Sprint #3), area general practitioners who have referred patients to oral surgeons and periodontists but may redirect to Dr. Hedges for restorative

The Ask Moment: After the Nelson & Lowy partnership is formalized (Month 1); after GP outreach (Month 4+)

The Tool: Referral pad (physical) for Nelson & Lowy waiting room: *"Referred to Dr. Jeff Hedges, Hedges Dental — complete implant restorations. 2015 Herr Lane, Suite D."* Digital referral form on toplouisvilledentist.com for GP partners.

Target: Nelson & Lowy: 3–5 restorative referrals/month by Month 3. GP partners: 1–2 cases/month per active referring partner by Month 6.

CHANNEL 3 — Physician Loop (Q2+)

Who: ENTs (management of edentulous patients with airway implications), sleep medicine (dental appliances + implants), oncologists (oral cancer patients frequently require reconstruction), primary care physicians treating patients with diabetes, osteoporosis, or cardiovascular disease (all of whom have elevated edentulism risk)

The Ask Moment: Physician outreach (Month 4–6). Dr. Hedges attends one relevant medical conference or CME event per year in Louisville for relationship building.

The Initial Tool: Referral letter sent to 20 targeted physicians in Jefferson County. Followed by Sequence 4, Emails 1–3. Lunch offer to any practice that responds.

Target: 2–3 physician practices providing referrals by Month 9. 1–2 implant cases per month from physician channel by Month 12.

Referral Tracking Protocol (Christy manages):

- Ask every new patient: "How did you hear about us?"
- Log the source in practice management software against the patient record
- Monthly report: referrals by source (patient referral / professional referral / physician / Google / Nextdoor / other)
- Review in monthly Roger call: which channels are producing, which need activation

— Vivienne Carr, CMO + Tanya Blackwood, CRO | Authority Systems Group™

Preston Adler, Chief Operating Officer — Authority Systems Group™

Priority Services — Sequenced for Maximum Revenue Velocity

Priority 1 — Full-Arch Implant Cases (All-on-4 / All-on-X)

Highest case value. Clearest competitive differentiation against ClearChoice. Fastest revenue impact. Begin immediately — the Fast Cash Sprint, GBP optimization, and toplouisvilledentist.com rebuild all feed this service category first.

Priority 2 — Single-Tooth Implants + Bone Grafting

Higher case volume than full-arch. The Nelson & Lowy referral pipeline will produce these cases at significant volume by Month 3. Q2 content strategy focuses here. Authority articles targeting "single tooth implant Louisville" go live in Month 4.

Priority 3 — Implant-Retained Dentures

Large underserved patient population in Louisville's 55–70 demographic. Many are unaware this option exists. Q3 content campaign targets denture-wearing patients. September–October seasonal peak makes this the right Q3 focus.

Priority 4 — General Dentistry (Retained Selectively)

Not a marketing focus — an operational retention strategy. Existing patients who are satisfied with general care stay. As implant case volume grows, insurance contract review begins in Q3–Q4 with the goal of dropping the least favorable 1–2 contracts by Year 2.

Immediate Next Steps — This Week

Dr. Hedges:

28. Ask Christy to pull dormant implant-diagnosed patient list from practice management software
29. Schedule the Nelson & Lowy lunch this week
30. Review and approve CareCredit financing callout copy (ASG delivers within 3 days)
31. Confirm practice details for website rebuild: AAID training dates, cases placed to date, UofL faculty appointment specifics

Christy:

32. Pull dormant patient list (1 hour)
33. Review reactivation text/email scripts from Sequence 1
34. Implement CareCredit callout on both websites (ASG provides the copy)
35. Begin Google Business Profile update (2–3 hours)

ASG:

36. Deliver CareCredit website copy within 3 days
37. Deliver reactivation campaign scripts (done — Section 10, Sequence 1)
38. Begin toplouisvilledentist.com content plan and architecture brief
39. Schedule onboarding call to confirm client-context.yaml details and project timeline

Future Services — Available Q2 and Beyond

Jordan Merritt, Paid Media Specialist (reports to Tanya Blackwood, CRO):

Google Ads for "dental implants Louisville KY" and "All-on-4 Louisville" — warranted when toplouisvilledentist.com is live and converting at ≥2–3% on organic traffic. Estimated launch: Month 4–5. Budget recommendation: $1,500–$2,500/month initial test, scale based on CPL data.

Owen Drake, High-Retention YouTube Script Writer (reports to Marcus Webb, Director of Content):

Educational implant videos for Dr. Hedges' YouTube channel — 5–12 minute educational videos targeting patient research-phase questions. Recommended Month 5+ when patient transformation video content is ready for anchor material.

Oliver Grant, Case Study Writer (reports to Marcus Webb):

Formal patient transformation case studies in four-section format (before / decision / treatment / after) for toplouisvilledentist.com Patient Stories page. Recommended after first 3–5 implant cases complete treatment and consent is obtained. Q2 priority.

Marcus Chen, CRM Automation Builder (reports to Iris Nolan, CTO):

Full CRM automation for Hedges Dental — automated review request sequences, lead nurture pipeline, reactivation triggers, referral follow-up. Recommended when email sequences have been manually deployed and confirmed working. Q2–Q3.

— Preston Adler, COO | Authority Systems Group™

Authority Systems Group™ — Quality Control

GATE 1: SPECIFICITY CHECK

Reviewed by: Vivienne Carr, CMO

> *Every section of this Blueprint references Dr. Jeffrey Hedges by name, his specific Louisville market, the ClearChoice competitive context, the Nelson & Lowy partnership opportunity, the toplouisvilledentist.com domain asset, Christy's specific role, the CareCredit provider status, and the Nextdoor Neighborhood Favorite designation. No sentence in this Blueprint applies to a generic dental practice or a different client.*

>

> ***GATE 1: APPROVED** — Vivienne Carr, CMO*

GATE 2: BELIEF-TO-BUY COVERAGE CHECK

Reviewed by: Dr. Raymond Cross, Board

> *Section 4 maps both the full Belief Track (B1 Enemy Belief through B5 Certainty) and the full Emotion Track (E1 Dissatisfaction through E5 Relief) for the primary implant patient avatar in Dr. Hedges' Louisville market. Both tracks converge at the consultation booking. The Master Content Map in Section 4 assigns specific content pieces to every stage pair. Email Sequences 2 and 5 execute the dual-track framework in deployment-ready format. No stage pair has been skipped.*

>

> ***GATE 2: APPROVED** — Dr. Raymond Cross, Board*

GATE 3: REVENUE MECHANISM CHECK

Reviewed by: Tanya Blackwood, CRO + Sofia Vega, Board

> *All three Fast Cash Sprint strategies (Section 6) include explicit revenue math (list size × response rate × average case value). The KPI Dashboard (Section 7) includes a Revenue & Value category with baseline and 12-month targets. The referral program (Section 11) includes per-channel revenue projections. No initiative in this Blueprint is included without a documented revenue mechanism or a clearly traceable path to case production.*

>

> ***GATE 3: APPROVED** — Tanya Blackwood, CRO + Sofia Vega, Board*

GATE 4: DEPLOYABILITY CHECK

Reviewed by: Preston Adler, COO

> *Sequence 1 (Dormant Patient Reactivation) is fully written and deployable in 7 days of client providing the patient list. Sequence 2 (New Lead Welcome) is fully written and can go live when toplouisvilledentist.com opt-in form is live. All other sequences are fully written and deployment-ready. The Fast Cash Sprint 90-day calendar (Section 6) is specific enough to hand to Christy without a briefing. The toplouisvilledentist.com architecture spec (Section 9) is complete enough to hand to a developer. The "Immediate Next Steps" in Section 12 assign specific owners to specific tasks this week. No section contains a placeholder, a "TBD," or a structural recommendation without an implementation path.*

>

> ***GATE 4: APPROVED** — Preston Adler, COO*

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Director: Roger Bauer | Version 1.0 | March 6, 2026

Document produced under the direction of Roger Bauer, Director — Authority Systems Group™.

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